MSC MARRIAGE AND FAMILY THERAPY

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MFT 715

Family Systems Theory

15 teaching weeks · 3 practicals · 0 worked cases · 7 figures

SEMESTER TWO

1.71 units · Prerequisite: MFT 702

> “This is the theoretical anchor of the programme.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 715 — Family Systems Theory

MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 702

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	**Family Systems Theory**	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. This is the theoretical anchor of the programme. Where MFT 702 surveys schools, this course interrogates the theory beneath them: cybernetics, general systems thinking, structure, communication, constructionism and their critics. You will finish able to explain precisely what you mean when you say a family is "a system".

How to use this book

The word "system" is used loosely in clinical settings. This course tightens it. Each week takes one theoretical tradition, states its claims precisely, applies it to a case, and then asks what it cannot explain. You are expected to leave with arguments, not slogans.

1. Course description

Family systems theory is not one theory but a family of theories sharing a commitment: **the unit of analysis is the relationship, and the whole is more than the sum of its parts**. The course traces that commitment from von Bertalanffy's general systems theory and the Macy conferences on cybernetics, through the Palo Alto communication school, Minuchin's structuralism, Bowen's multigenerational theory, and the Milan group, to social constructionism, critical and feminist theory, and contemporary complexity and network perspectives.

2. Learning outcomes

#	Outcome	Assessed
LO1	Explain the core propositions of general systems theory and cybernetics	Exam
LO2	Analyse a family using at least four theoretical traditions	Case analysis
LO3	Critique each tradition, including its cultural and political assumptions	Essay
LO4	Distinguish first- and second-order cybernetics and demonstrate reflexivity	Viva
LO5	Apply constructionist ideas to meaning-making in a family	Case analysis
LO6	Evaluate the evidence for systemic propositions	Exam
LO7	Construct a coherent theoretical position of your own	Position paper

3. Assessment

Component	Weight	Due
Theoretical analysis of a case using four traditions (2,500 words)	35%	Week 9
Essay: critique of a systemic concept (2,000 words)	25%	Week 13
Examination	25%	Week 16

Component	Weight	Due
Position paper and viva (1,000 words)	15%	Week 15

4. The fifteen-week plan

Week	Topic	Practical
1	The systemic turn: what it was and what it answered	Compare two accounts of one case
2	General systems theory: wholes, boundaries, equifinality	Systems vocabulary drill
3	Cybernetics I: feedback, control, homeostasis	Feedback mapping
4	Cybernetics II: first and second order; reflexivity	Reflexivity exercise
5	Communication theory: axioms, punctuation, paradox	Transcript analysis
6	Structural theory: organisation, hierarchy, boundaries	Structural mapping
7	Multigenerational theory: anxiety, triangles, differentiation	Triangle mapping
8	Milan and meaning: beliefs, premises, circularity	Circular questioning
9	**Theoretical analysis due**; constructivism and social constructionism	Meaning-making interview
10	Postmodern and narrative theory: knowledge and power	Texts and stories exercise
11	Critical, feminist and anti-racist systems theory	Power audit of a model
12	Attachment and neuroscience meet systems theory	Integration diagram
13	**Essay due**; critiques and limitations of systems thinking	Debate
14	Complexity, networks and the contemporary field	Network map of a family
15	Your theoretical position	Viva

Week 1 — The systemic turn

What changed. Before the systemic turn, the family was understood as the environment of an individual patient. After it, the family became the unit of treatment, and individual symptoms became functions of relationship patterns.

What it answered. Three problems the individual model struggled with: Why does a symptom appear in a particular child? Why does the family resist change (homeostasis)? Why does the same individual behave differently in different relationships?

What it risked. Dissolving the person into the system; treating "the family" as a homogeneous actor; ignoring power, gender, class and history; and producing descriptions that could not be falsified.

PRACTICAL

Take one case. Write it twice: once as an individual formulation (as a CBT or psychodynamic clinician would) and once as a systemic one. Then write a paragraph on what each version makes visible and invisible.

Week 2 — General systems theory

Von Bertalanffy's propositions, translated for clinicians:

Proposition	Clinical meaning
A system is a set of elements in interaction	The family is defined by its interactions, not its members
The whole is more than the sum of its parts	The family has properties no member has alone
Systems are open or closed to their environment	A family that refuses outside contact becomes closed and rigid
Systems have boundaries	Who is inside, who is outside, and how permeable
Systems are hierarchical	Subsystems nested within larger systems
Equifinality	Different starting points can reach the same end state
Equipotentiality	Similar starting points can reach different end states
Negative entropy	Systems import energy to counter disorder (information, support)

Equifinality and equipotentiality are the most useful and most neglected. Equifinality explains why two very different families produce the same symptom; equipotentiality explains why two similar families produce very different outcomes. Together they demolish simple causal stories.

Example. Two children are referred for school refusal: one in a family with high conflict and one in a family with high achievement pressure. Equifinality explains the identical presentation; equipotentiality explains why one family recovers in four sessions and the other takes two years.

Week 3 — Cybernetics I

Feedback. A system regulates itself by monitoring its own output and adjusting its input.

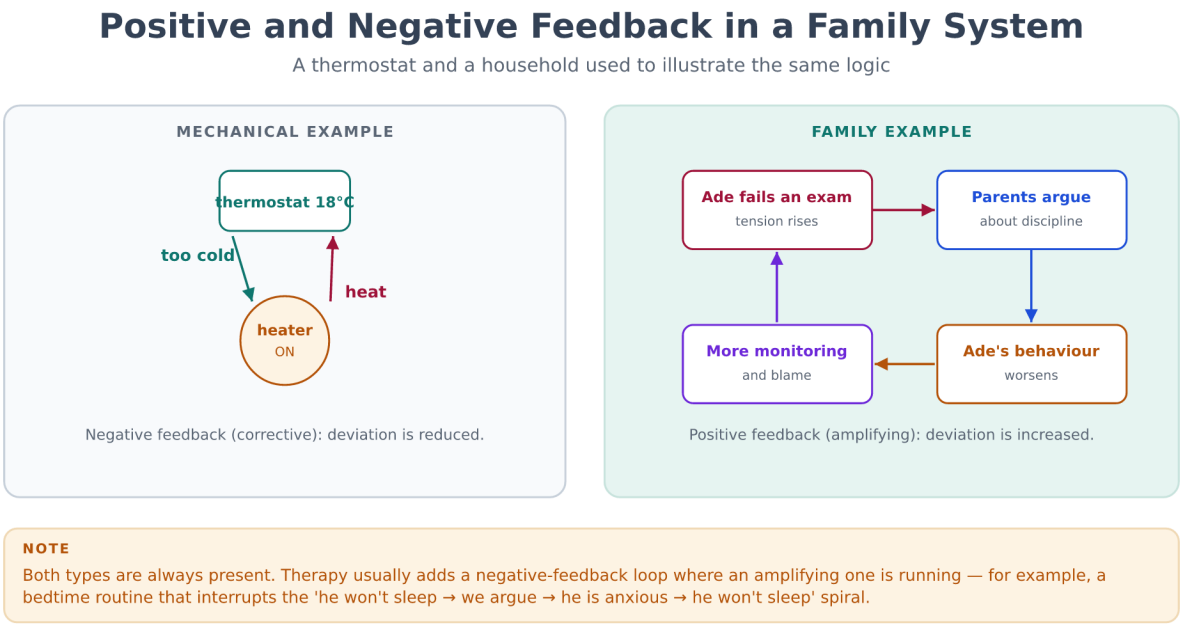


Figure 3.1 — Negative and positive feedback in a machine and in a family

- **Negative feedback** corrects deviation: the thermostat, the bedtime routine, the family that de-escalates when voices rise.
- **Positive feedback** amplifies deviation: the microphone screech, the escalation, the panic spiral.

Homeostasis. The system's tendency to maintain its organisation. **Morphostasis** maintains; **morphogenesis** changes. A family can be stably unhappy: the symptom is one of the mechanisms that holds the system together.

The clinical insight. Attempts to change a system are themselves inputs. If the family's equilibrium depends on the symptom, your intervention will produce a counter-move. Predicting the counter-move — and sometimes prescribing it — is the strategic tradition's contribution.

Calibration example. A family's daughter has "tantrums" that end a 45-minute argument between her parents. The tantrum is negative feedback for the marital system (it restores calm) and positive feedback for the child (it works). Two feedback systems, one event.

> **PRACTICAL**
> Map both feedback loops in a disguised case: one for each "layer" (the couple system and the child's behaviour). Show how the same event serves both.

Week 4 — Cybernetics II: first and second order

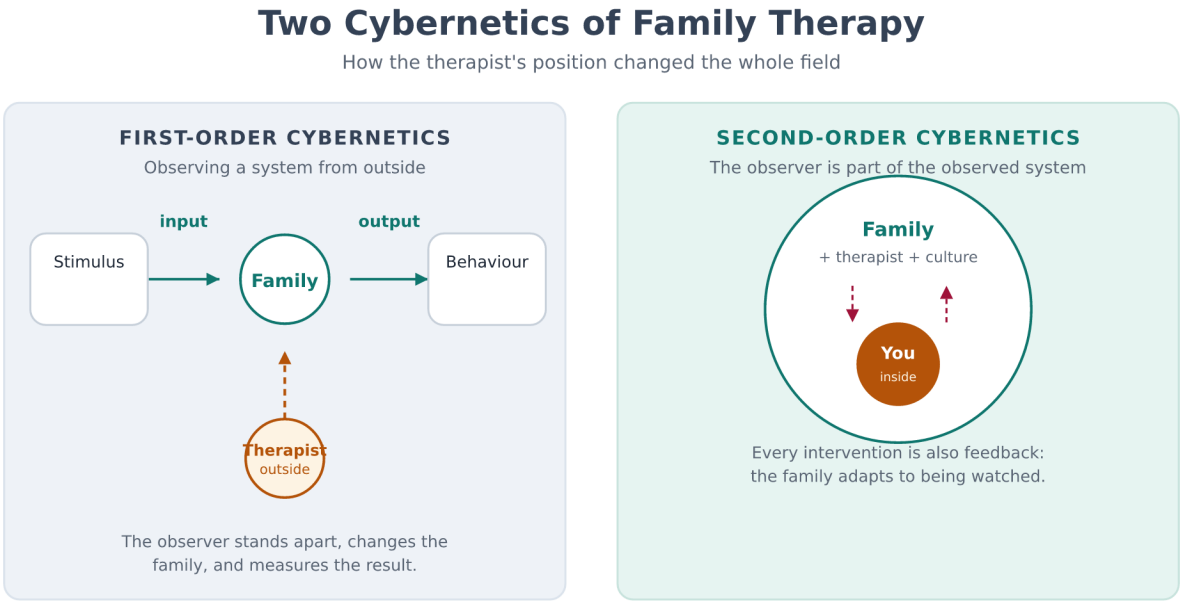


Figure 4.1 — First- and second-order cybernetics: where the therapist stands

First-order cybernetics. The observer is outside the system; interventions are inputs; the aim is control of the system's output. This produced the heroic era: directive, strategic, clever.

Second-order cybernetics. The observer is part of the observed system; distinctions are drawn by an observer; knowledge is provisional. This produced curiosity, the reflecting team, and the therapist's accountability for their own effect.

The clinical consequences.

First order	Second order
"What intervention will fix this family?"	"What am I noticing, and what am I blind to?"
The family is the object of change	The family and the therapist co-construct change
Neutrality is possible	Position is unavoidable; transparency is required
Expertise is the therapist's	Expertise is distributed

Reflexivity is not self-indulgence. It is the discipline of accounting for your own contribution to the clinical picture: which family member you find easier to like, which story you believed first, which data you stopped collecting once you had a hypothesis.

Week 5 — Communication theory



Figure 5.1 — Watzlawick's five axioms of communication

The five axioms (Watzlawick, Beavin & Jackson).

1. One cannot not communicate.
2. Every message has a content and a relationship level.
3. Communication is punctuated into sequences by participants.
4. Messages are digital (words) and analogical (analogue).
5. Relationships are symmetrical or complementary.

Double bind hypothesis (Bateson): a person receives contradictory messages at different levels, cannot comment on the contradiction, and cannot leave the relationship. The hypothesis was weakened as an explanation of schizophrenia, but its clinical descendant — recognising contradictory, un-commentable communications in families — remains useful.

Critique. Communication theory could become mechanistic (families as machines exchanging messages) and could pathologise families of people with psychosis. It also tended to describe "systems" without the power differences that determine who gets to define what is communicated.

> **PRACTICAL**
> Transcript analysis: take ten lines of a couple's argument and identify punctuation, relationship-level messages, and symmetrical/complementary moves.

Week 6 — Structural theory

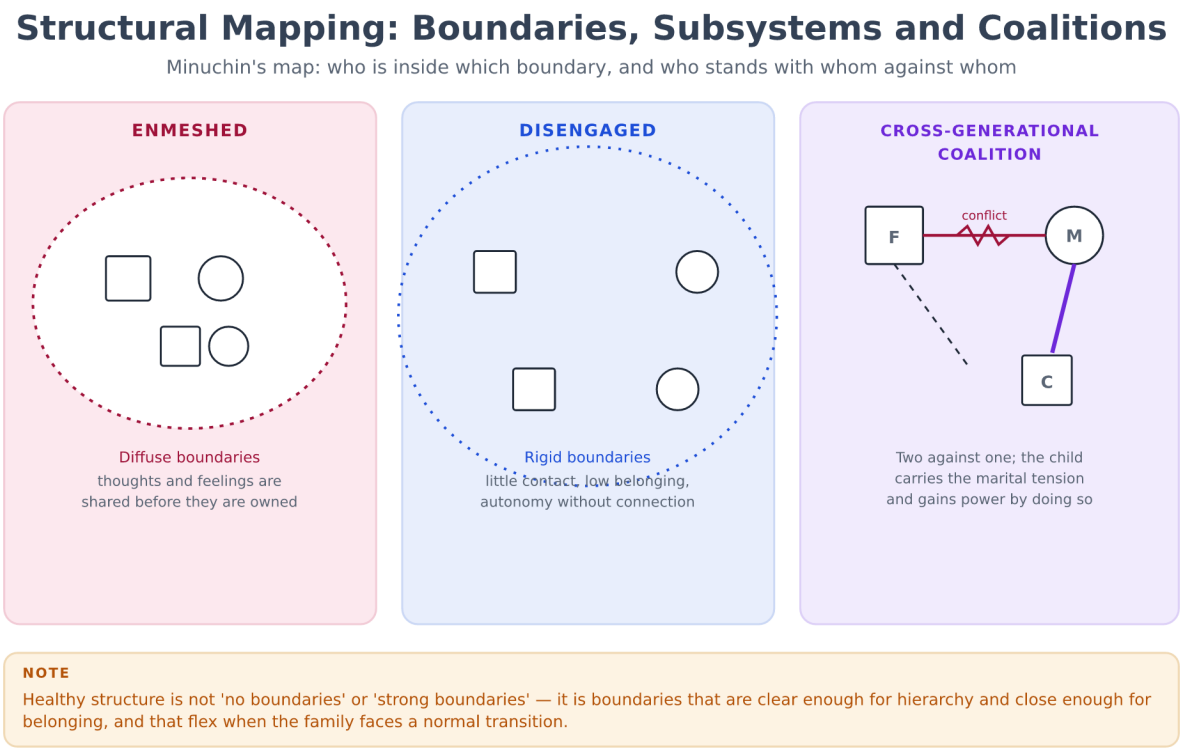


Figure 6.1 — Structural mapping: enmeshed, disengaged and cross-generational coalition

Core propositions (Minuchin).

1. The family's structure is the invisible set of rules that govern transactions.
2. The structure organises the family into subsystems (spousal, parental, sibling) separated by boundaries.
3. Symptoms are best understood as the family's response to its structure, and sometimes as the only way the structure can remain stable.
4. Change in structure produces change in experience.

The evaluation criteria for a structure. Boundaries must be clear (not diffuse, not rigid); hierarchy must be appropriate to stage and culture; and subsystems must have enough autonomy to function and enough connection to belong.

Power and structure. Structural theory was criticised for assuming a normative hierarchy (executive parents, subordinate children) and for not asking how that hierarchy is upheld. Contemporary structural practice inserts the questions: whose authority is legitimate in this family, and who bears the cost of it?

Week 7 — Multigenerational theory

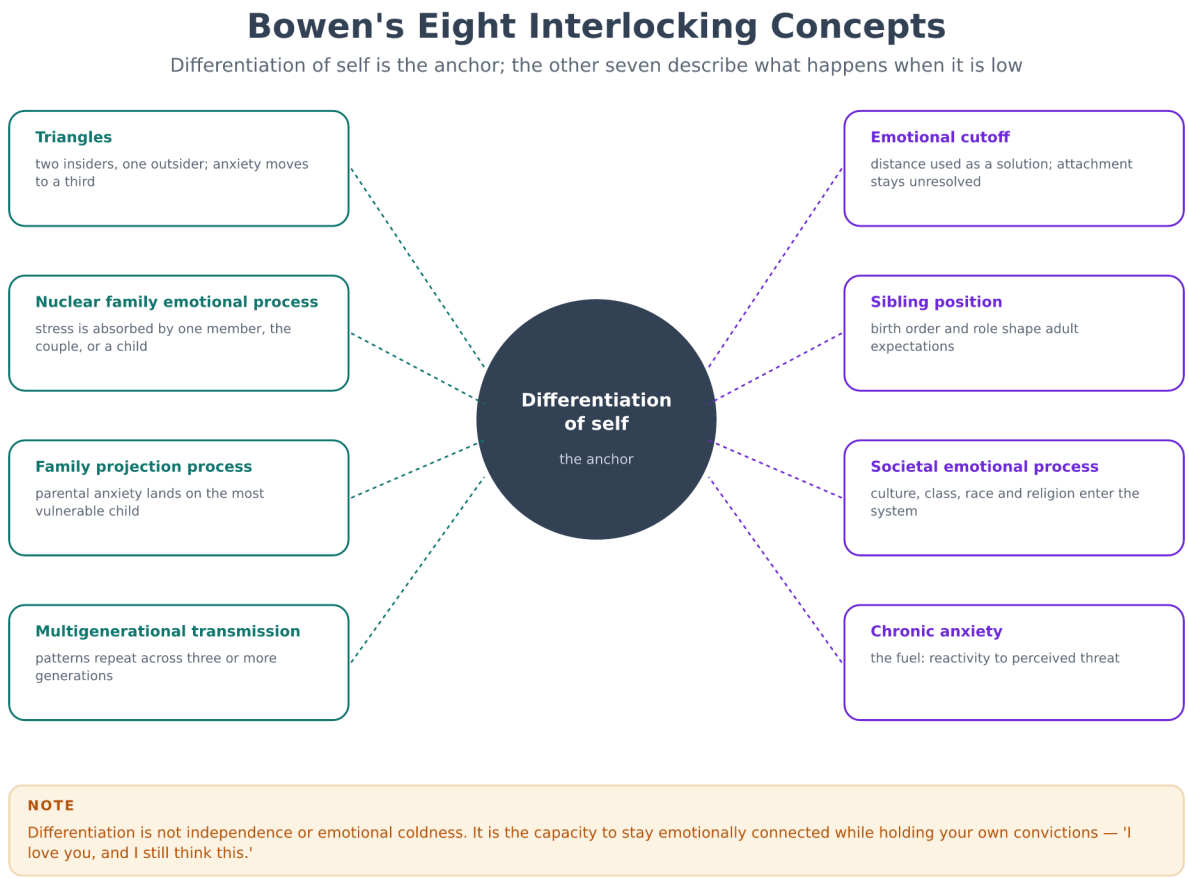


Figure 7.1 — Bowen's eight interlocking concepts

Core propositions (Bowen).

1. The family is an emotional unit; anxiety is its most important variable.
2. Differentiation of self is the capacity to remain connected while thinking independently.
3. Anxiety is managed in one of four ways: conflict, distance, projection to a child, or dysfunction in one member.
4. Triangles distribute anxiety and stabilise dyads.
5. Patterns are transmitted across generations; symptomatic children are often the most triangulated.

The theory's strength is time: it offers an account of why now, by locating the present pattern in several generations of adaptation to war, migration, loss and religion.

The theory's weakness is its insufficient account of power, material constraint and culture. A woman cannot always "differentiate" out of an arranged marriage; a family cannot always "stay out of triangles" when migration has severed the network.

Week 8 — Milan and meaning

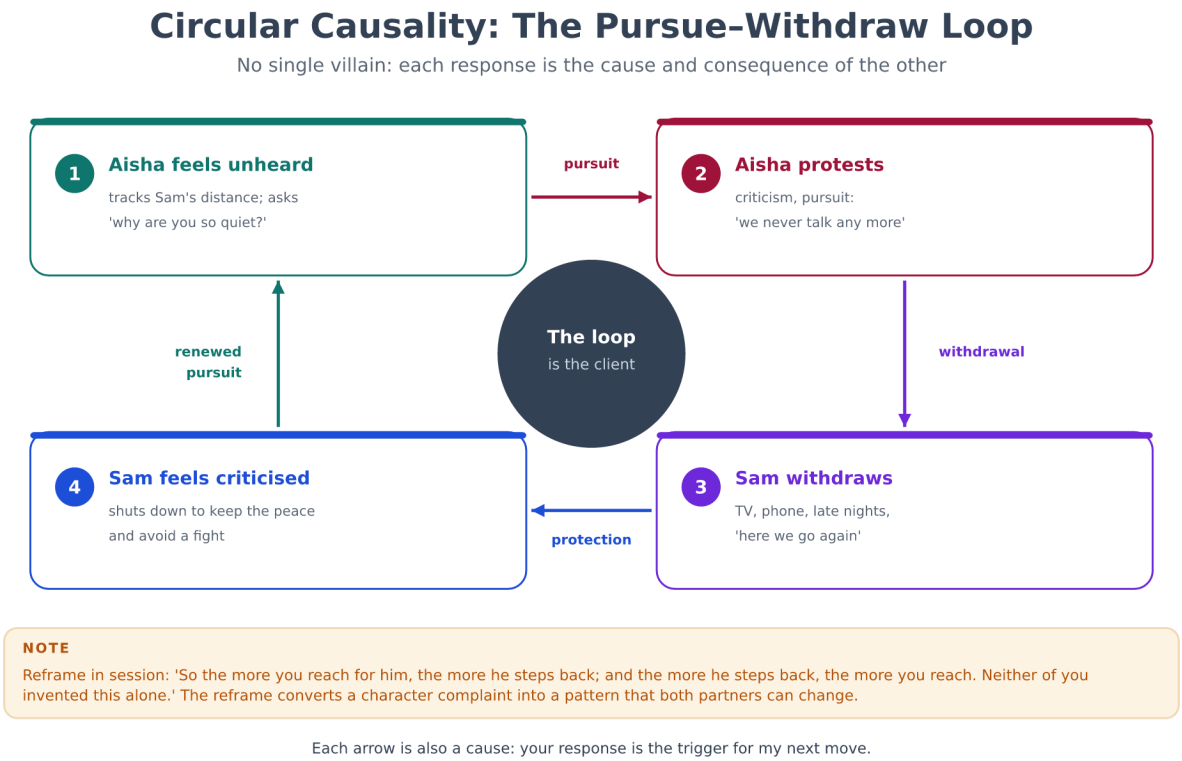


Figure 8.1 — Circular causality in the consulting room

Core propositions (Palazzoli, Boscolo, Cecchin, Prata).

1. The family is a system of meanings (premises) that organise behaviour.
2. The therapist forms hypotheses and tests them with circular questions.
3. Neutrality is a stance of curiosity towards all members, not indifference.
4. Positive connotation frames all behaviour as serving the system's coherence.
5. Interventions disturb the system by changing premises, not by demanding behaviour change.

The evolution. Classical Milan used teams, rituals and directives; later Milan (and the reflecting team) abandoned directive intervention and emphasised multiple perspectives and the therapist's own openness.

Critique. Neutrality became indefensible in cases of violence and abuse; circular questioning can become an elegant evasion; and the intellectual register excluded many families.

Week 9 — Constructivism and social constructionism

Constructivism (Maturana, von Glasersfeld): each person constructs a model of reality from their own experience; we cannot access an objective reality directly. Clinically: take each family member's account seriously, and stop trying to establish "the truth".

Social constructionism (Gergen, Shotter): meanings are produced between people, in language and in culture, and change with context. Clinically: the problem, the roles and the identities are made in talk, and so they can be made differently.

The distinction matters. Constructivism risks relativism and individualism (everything is "just your view"); social constructionism locates meaning in relationships and power, which makes it compatible with the critical and cultural traditions.

Implication for practice. The therapist's formulation is one description among several. Offering it humbly — "here is a story I am beginning to see; how does it look to you?" — is both a constructionist technique and a mark of respect.

Week 10 — Narrative and postmodern theory

Narrative (White & Epston). Stories organise experience; dominant stories make alternatives invisible; the task is to externalise the problem and thicken a preferred story.

Postmodern influences. Foucault on discourse and power-knowledge; Derrida on deconstruction. The clinical translation: question the "official" account, ask who benefits from it, and look for what it silences.

The reflexivity of narrative therapy is unusual: the therapist is "decentred but influential", and the client is the author. This is an ethical stance as much as a technique.

Critique. The linguistic emphasis can under-serve families with limited literacy or in acute crisis; the evidence base, while growing, is smaller than the cognitive-behavioural tradition's.

Week 11 — Critical, feminist and anti-racist systems theory

The feminist critique (Hare-Mustin 1978; Goldner; Walters) made four claims that changed the field:

1. "Circular" descriptions conceal power asymmetries.
2. "Neutrality" in the face of violence colludes with the perpetrator.
3. The gendered division of labour, not "the family system", is often the object of the wife's complaint.
4. Models that prize individuation encode a male norm.

Anti-racist and postcolonial theory extend the critique: whose family structures are treated as healthy? Which knowledge counts as evidence? How do services reproduce the state's relationship to marginalised families?

Clinical translation.

- Ask about power explicitly, in every assessment.

- Never use systemic language to make a victim responsible for the violence she receives.
- Place race, migration, class and religion inside the system, not outside it.

Week 12 — Attachment and neuroscience in systems theory

Two imports that have reshaped the field.

1. **Attachment theory** gives systemic thinking a developmental mechanism: the couple and parent–child systems are organised by attachment strategies, and those strategies are formed in relationships.
2. **Neuroscience** gives systemic thinking a physiology: co-regulation, the window of tolerance, mirroring and stress physiology explain how relationships change bodies.

The integration. "Systems" are not abstract patterns; they are embodied patterns between nervous systems. A couple's pursue–withdraw cycle is simultaneously a communication pattern, an attachment strategy, and a sequence of autonomic states.

Critique of the imports. Neuroscience is often oversold in clinical texts; "mirror neurons" and "the right brain" are frequently invoked without evidence. Use physiology where it is well established (stress, regulation, sleep, trauma) and keep the metaphors labelled as metaphors.

Week 13 — Critiques and limitations

Critique	Substance	Response
The system has no agent	"The family did it" hides who actually did what	Locate action; ask who decided
Power is under-theorised	Systemic descriptions can neutralise abuse	Explicit power analysis
Culture is externalised	"The system" was a Western two-parent family	Cultural and critical turns
Evidence is thin	Many systems concepts are unfalsifiable	Operationalise and test
The therapist disappears	Neutral observer stance	Second-order reflexivity
Complexity obscures action	Endless circular description delays intervention	Use the simplest map that guides action

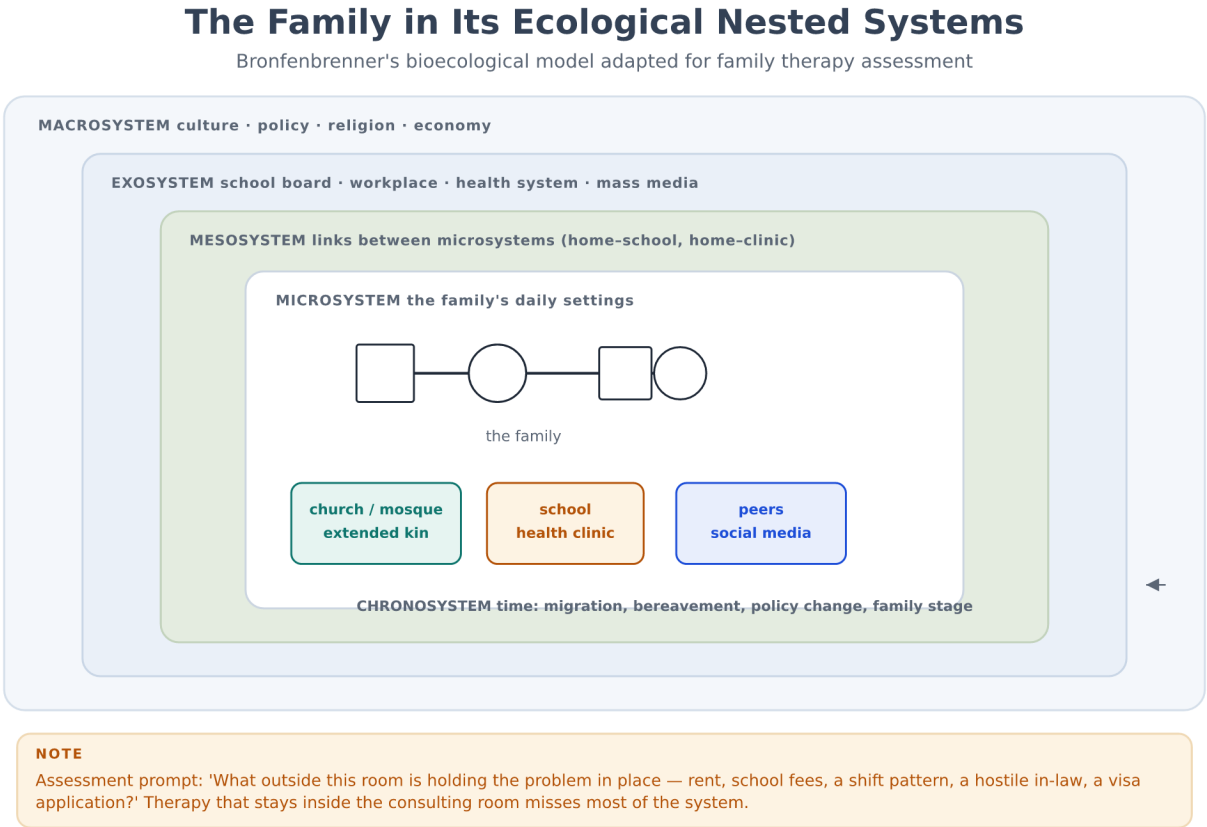
The mature position. Systems theory is a disciplinary sensibility, not a complete theory. It tells you where to look (between people, across generations, in context) and what to resist (single villains and simple causes). It does not, by itself, tell you what to do.

Week 14 — Complexity, networks and the contemporary field

Complexity theory. Families are non-linear, self-organising systems: small differences can produce large divergence; cause and effect are not proportional; and feedback loops are the rule. This fits clinical experience better than linear models, but it resists prediction.

Network thinking. Contemporary research treats families as networks of relationships, with density, centrality and clustering measurable. Who is central? Who is isolated? Who is the only link between two clusters? This is a formalised genogram, and it is improving our understanding of family structure and resilience.

Ecological models. Bronfenbrenner's nested systems (Figure 7 in MFT 701) place the family inside institutions, culture and time — a corrective to the field's tendency to treat the family as self-contained.



NOTE

Assessment prompt: 'What outside this room is holding the problem in place — rent, school fees, a shift pattern, a hostile in-law, a visa application?' Therapy that stays inside the consulting room misses most of the system.

Figure 14.1 — The family inside nested ecological systems

Week 15 — Your theoretical position

Write 1,000 words and defend it in ten minutes:

1. What is the unit of analysis you will work with, and why?
2. What do you believe about causation in families?

3. What is the therapist's position, and how will you manage your own power?
4. Which systemic concept do you find least defensible, and what do you use in its place?
5. What would make you abandon your position?

Glossary

Autopoiesis — the self-producing organisation of living systems. **Boundary** — the rules governing participation and exchange between systems. **Circularity** — reciprocal causation within a system. **Cybernetics** — the study of control and communication in systems. **Differentiation of self** — see Bowen. **Equifinality** — different paths leading to the same end state. **Equipotentiality** — similar starting points leading to different end states. **Homeostasis / morphostasis** — the maintenance of system organisation. **Morphogenesis** — change in system organisation. **Punctuation** — the marking of a sequence's beginning; the source of blame. **Reflexivity** — the therapist's accountability for their own participation. **Second-order cybernetics** — cybernetics of observing systems. **Structure** — the repetitive patterns of transaction that organise a family.

Self-assessment

Multiple choice

1. Equifinality means: (a) same cause, same outcome (b) different paths, same outcome (c) same paths, different outcomes (d) no causation

2. Homeostasis in a family refers to: (a) change (b) the maintenance of existing organisation (c) hierarchy (d) communication

3. Second-order cybernetics holds that the observer: (a) is outside the system (b) is part of the observed system (c) is irrelevant (d) is objective

4. "One cannot not communicate" is: (a) axiom 1 (b) axiom 3 (c) axiom 5 (d) a double bind

5. Punctuation is clinically important because it: (a) describes memory (b) assigns blame for a sequence (c) measures cohesion (d) is a diagnostic criterion

6. The feminist critique of family therapy argued that: (a) systems are too complex (b) circular descriptions conceal power asymmetries (c) hierarchy is unnecessary (d) therapy should be individual

7. Positive connotation is associated with: (a) structural theory (b) Milan (c) Bowen (d) behavioural therapy

8. A closed system in family terms is one that: (a) has clear boundaries (b) has limited exchange with its environment and becomes rigid (c) is disengaged (d) is enmeshed

9. Social constructionism locates meaning in: (a) the individual brain (b) relationships, language and culture (c) genetics (d) behaviour

10. The most defensible use of systems theory is as: (a) a complete predictive model (b) a disciplinary sensibility that shapes where you look (c) a diagnosis (d) a legal framework

Answer key

1 b · 2 b · 3 b · 4 a · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. Define equifinality and equipotentiality and give one clinical example of each.
2. Distinguish negative and positive feedback with a family example.
3. What are the three criteria for evaluating a family's structure?
4. State the feminist critique of "circular" formulations in four sentences.
5. What is reflexivity, and why is it a clinical skill?

Essay (choose one, 2,000 words)

1. "Systems theory tells you where to look but not what to do." Discuss.
2. Critically evaluate the concept of homeostasis in families where violence is present.
3. Compare Bowen's and Minuchin's accounts of how problems develop, and assess which is better supported.

Further reading

- von Bertalanffy, L. (1968). General System Theory. Braziller.
- Watzlawick, P., Beavin, J., & Jackson, D. (1967). Pragmatics of Human Communication. Norton.
- Minuchin, S. (1974). Families and Family Therapy. Harvard University Press.
- Bowen, M. (1978). Family Therapy in Clinical Practice. Aronson.
- Becvar, D. S., & Becvar, R. J. (2018). Family Therapy: A Systemic Integration (9th ed.). Pearson.
- Gergen, K. J. (2009). An Invitation to Social Construction (2nd ed.). Sage.
- Hare-Mustin, R. T. (1978). A feminist approach to family therapy. Family Process, 17(2), 181–194.

End of Semester Two. Next: MFT 716 — Communication Skills in Therapy